WEAVE

CENTRE FOR INTEGRATIVE PSYCHIATRY

WEAVE PSYCHOTHERAPY — VOL. 6

Third-Wave Therapies

DBT, Schema Therapy, ACT, MBCT, and Compassion-Focused Therapy

DBT Skills · Schema Modes · 18 EMSs · ACT Hexaflex · MBCT · Biosocial Model · Imagery Rescripting

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Deep Study

Third-Wave Therapies — Weave Psychotherapy Vol. 6

D1: Third-Wave Therapies — Deep Study

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1. WHAT IS THE "THIRD WAVE"?

1.1 Hayes' Three Waves of Behaviour Therapy

Steven Hayes (2004) proposed a taxonomy of behaviour therapy's evolution:

First Wave — Classical Behaviourism (1950s–1970s). Rooted in Pavlov, Skinner, and Wolpe. Therapy meant modifying observable behaviour through conditioning: systematic desensitisation, token economies, exposure and response prevention. Internal experience was either irrelevant or inaccessible. The therapist was a technician applying learning principles.

Second Wave — Cognitive Revolution (1970s–1990s). Beck, Ellis, and Meichenbaum brought internal cognitions centre stage. Dysfunctional thoughts cause emotional disturbance; therapy means identifying and changing distorted cognitions. Cognitive restructuring, thought records, Socratic questioning, and guided discovery became the core methods. The therapist was a scientist helping the patient test hypotheses.

Third Wave — Contextual and Process-Based Approaches (1990s-present). Rather than changing the content of thoughts, third-wave therapies change the relationship to thoughts. Key features:

- **Acceptance over control** — emotions and thoughts are not enemies to be eliminated but experiences to be observed and tolerated
- **Mindfulness** — present-moment, nonjudgmental awareness as a core skill, not just a technique
- **Values and meaning** — behaviour is guided by personal values, not just symptom reduction
- **Therapeutic relationship as active ingredient** — not merely the vehicle for delivering techniques but itself a mechanism of change
- **Metacognition** — stepping back to observe one's own thinking processes rather than engaging with thought content
- **Dialectics** — holding opposing truths simultaneously; synthesis rather than linear logic
- **Contextual sensitivity** — behaviour understood in its context, not as inherently "dysfunctional"

> EXAM PEARL
>
> The three waves are not sequential replacements — each wave built on and incorporated the previous. DBT uses behavioural skills training (first wave) + cognitive modification (second wave) + acceptance and mindfulness (third wave). Schema Therapy integrates cognitive restructuring (second wave) with experiential techniques, attachment theory, and limited reparenting (third wave).

> EXAM STRATEGY
>
> When asked to classify a therapy as "third wave," look for these distinguishing features: acceptance-based stance, mindfulness component, emphasis on the therapeutic relationship as a change mechanism, values orientation, and metacognitive or process-level intervention. If a therapy only uses cognitive restructuring and behavioural experiments, it is second wave.

1.2 The Major Third-Wave Therapies

Therapy	Founder(s)	Year	Core Innovation
Dialectical Behavior Therapy (DBT)	Marsha Linehan	1993	Dialectics: acceptance + change; skills-based
Schema Therapy (ST)	Jeffrey Young	1990/2003	Early Maladaptive Schemas; limited reparenting
Acceptance and Commitment Therapy (ACT)	Steven Hayes	1999	Psychological flexibility; defusion; values
Mindfulness-Based Cognitive Therapy (MBCT)	Segal, Williams, Teasdale	2002	Mindfulness for depression relapse prevention
Compassion-Focused Therapy (CFT)	Paul Gilbert	2005	Three emotion regulation systems; compassionate mind
Functional Analytic Psychotherapy (FAP)	Kohlenberg & Tsai	1991	In-session behaviour as target of change
Metacognitive Therapy (MCT)	Adrian Wells	2009	Metacognitive beliefs; detached mindfulness
Behavioural Activation (BA)	Martell, Addis, Jacobson	2001	Context, not cognition; valued action in depression

> EXAM PEARL
>
> Not all authorities classify Schema Therapy as "third wave." Young developed it as an extension of CBT for characterological patients. However, its emphasis on the therapeutic relationship (limited reparenting), experiential techniques (imagery rescripting, chair work), and attachment-based conceptualisation place it firmly in the third-wave tradition. Most contemporary texts include it.

2. DIALECTICAL BEHAVIOR THERAPY (DBT)

2.1 The Biosocial Model

DBT explains chronic emotional dysregulation through a transactional model:

Biological vulnerability — three features:

- **High emotional sensitivity** — emotions triggered easily and by subtle cues
- **High emotional reactivity** — intense emotional responses once triggered
- **Slow return to baseline** — emotional arousal persists for hours, not minutes

Invalidating environment — consistently communicates that internal experiences are wrong, inappropriate, or an overreaction. Three types:

1. Punishing or dismissing valid emotional expression ("Stop crying, there's nothing to cry about")
2. Intermittently reinforcing only extreme emotional displays (shapes escalation)
3. Oversimplifying solutions ("Just say no," "Just try harder")

The transaction between biological vulnerability and environmental invalidation produces: emotional dysregulation, self-invalidation, behavioural dysregulation (impulsivity/avoidance), interpersonal dysregulation, and identity confusion.

EXAM PEARL

The biosocial model is transactional, not linear. The child's emotional sensitivity shapes parental responses; parental invalidation worsens the child's dysregulation; escalation cycles establish patterns. Neither biology nor environment alone is "the cause." This is the biosocial transaction.

CLINICAL ANCHOR

In clinical practice, presenting the biosocial model to patients is a powerful de-shaming intervention. It says: "Your problems are not your fault (biology + environment), but they are your responsibility to solve." This captures the central dialectic of DBT.

2.2 Dialectical Philosophy

DBT is grounded in three principles of dialectical philosophy:

1. **Interrelatedness** — everything is connected; a change in one part of the system affects all other parts
2. **Polarity** — reality is composed of opposing forces (thesis and antithesis); truth lies in the synthesis
3. **Continuous change** — the only constant is change; both patient and therapist are always evolving

The fundamental dialectic in DBT: **acceptance and change**. The patient is doing the best they can AND needs to do better. The therapist validates the patient's suffering AND pushes for new behaviour. Acceptance without change is stagnation. Change without acceptance is invalidation.

EXAM PEARL

Three dialectical dilemmas in standard DBT (Linehan): (1) Emotional vulnerability vs. self-invalidation, (2) Active passivity vs. apparent competence, (3) Unrelenting crisis vs. inhibited experiencing. Each generates secondary treatment targets.

EXAM PEARL

Three additional adolescent-specific dialectical dilemmas (Rathus & Miller): (1) Excessive leniency vs. authoritarian control, (2) Normalising pathological behaviours vs. pathologising normative behaviours, (3) Forcing autonomy vs. fostering dependence.

2.3 Four Treatment Modes

Mode	Function	Setting
Individual therapy	Motivational; applies skills to specific problems; processes diary card; follows target hierarchy	Weekly, 1 hour
Skills group	Skills acquisition and strengthening; teaching new capabilities	Weekly, 2–2.5 hours
Phone coaching	Skills generalisation to real-life crises; brief, skill-focused calls	As needed, between sessions
Consultation team	Therapy for the therapists; prevents burnout, maintains fidelity; treats therapists' own dialectical dilemmas	Weekly, 1–2 hours

EXAM PEARL

The four modes serve four distinct functions and must not be conflated. Individual therapy is for motivation and problem-solving; skills group is for capability. A patient cannot call the individual therapist to "process" emotions — phone coaching is specifically for "help me use a skill right now." The consultation team is not supervision — it is therapy for the therapist within a DBT framework.

> CLINICAL ANCHOR
>
> In adolescent DBT (Rathus & Miller), the skills group becomes a multifamily group — parents attend alongside teens. This directly addresses the invalidating environment by teaching parents the same skills and validation principles.

2.4 Target Hierarchy

The target hierarchy governs what is addressed in individual therapy sessions, in strict order:

Stage 1 Individual Therapy Target Hierarchy:

1. **Life-threatening behaviours** — suicidal ideation, self-harm, homicidal ideation
2. **Therapy-interfering behaviours** — missed sessions, non-compliance, therapist burnout
3. **Quality-of-life-interfering behaviours** — substance use, housing instability, eating disorders, interpersonal chaos
4. **Increasing behavioural skills** — generalising skills from group to daily life

Stage 1 Skills Training Target Hierarchy:

1. Decrease behaviours likely to destroy therapy
2. Increase skills acquisition, strengthening, and generalisation

> EXAM STRATEGY
>
> If an exam question describes a patient who missed sessions, used substances, and had suicidal thoughts this week, the correct answer for what to address first is always suicidal behaviour (life-threatening > therapy-interfering > quality-of-life). The hierarchy is absolute.

2.5 Stage Model (Stages 1-4)

Stage	Focus	Goal
Pretreatment	Orientation, assessment, commitment	Informed consent; mutual agreement to proceed
Stage 1	Behavioural stabilisation	Attain basic capacities — safety, control
Stage 2	Emotional processing	Reduce post-traumatic stress; exposure for trauma
Stage 3	Ordinary life problems	Increase self-respect; pursue individual goals
Stage 4	Completeness	Capacity for sustained joy; spiritual fulfilment

EXAM PEARL

Most DBT manuals and research address Stage 1 only. Stage 2 incorporates prolonged exposure techniques. Stage 4 was inspired by Linehan's Zen practice and addresses the existential dimension — moving from "a life worth living" to "a life experienced as complete."

2.6 DBT Skills by Module

Core Mindfulness

Skill Category	Skills	Description
Three States of Mind	Emotional Mind, Reasonable Mind, Wise Mind	Wise Mind = synthesis of emotion and reason; intuitive knowing
"What" Skills	Observe	Notice internal/external experience without words; "Teflon mind"
Describe	Put words on observations; stick to facts, no interpretations	
Participate	Throw yourself fully into the current activity; "in the zone"	
"How" Skills	Non-judgmentally	Notice without evaluating as good/bad; replace judgments with descriptions
One-mindfully	Do one thing at a time; when mind wanders, return gently	
Effectively	Do what works; let go of "right" vs. "wrong"; focus on goals	

> EXAM PEARL
>
> Mindfulness is not a separate module taught once — it is the foundation module, repeated before every other module begins. You cannot use any DBT skill without first noticing you need it. Mindfulness is the "gateway skill."

Distress Tolerance

Crisis Survival Skills:

Skill	Acronym/Name	Components
TIPP	Temperature, Intense exercise, Paced breathing, Progressive relaxation	Rapid physiological downregulation; activates PNS; for "red zone" distress
STOP	Stop, Take a step back, Observe, Proceed mindfully	Impulse control; prevents acting on the first urge
Wise Mind ACCEPTS	Activities, Contributing, Comparisons, Emotions, Pushing away, Thoughts, Sensations	Distraction techniques; done mindfully
Self-Soothe	Six senses: Vision, Hearing, Touch, Smell, Taste, Movement	Sensory self-comfort
IMPROVE the Moment	Imagery, Meaning, Prayer, Relaxation, One thing, Vacation, Encouragement	In-the-moment coping enhancers
Pros and Cons	Weigh pros/cons of acting on urge vs. using skills	Label each as short-term or long-term

Reality Acceptance Skills:

Skill	Description
Radical Acceptance	Full acknowledgment of reality as it is; acceptance does not equal approval; Pain + Non-acceptance = Suffering
Turning the Mind	Repeated, moment-by-moment choice to accept reality; acceptance is not a single decision
Willingness vs. Willfulness	Willingness = doing what the situation requires; Willfulness = refusing to engage with reality
Half-Smile	Subtle relaxation of facial muscles; behavioural cue for acceptance

EXAM PEARL

TIPP skills work as fast as dysfunctional behaviours (self-harm, substance use) but without the negative consequences. The Temperature skill activates the mammalian dive reflex: cold water on the face triggers the vagus nerve, rapidly reducing heart rate. This is parasympathetic activation — the physiological mechanism is identical to that targeted by vagal nerve stimulation.

CLINICAL ANCHOR

Radical acceptance is not agreeing that bad things are acceptable. It is acknowledging that they happened. Pain is unavoidable; suffering is pain multiplied by non-acceptance. "This should not have happened to me" is the non-acceptance that converts pain into prolonged suffering.

Emotion Regulation

Skill	Acronym/Name	Components
ABC PLEASE	Accumulate positives, Build mastery, Cope ahead, treat PhysicaL illness, balance Eating, Avoid mood-altering substances, balance Sleep, get Exercise	Vulnerability reduction; proactive emotional management
Check the Facts	—	Is the emotion fitting the facts? Are interpretations accurate?
Opposite Action	—	Act opposite to the emotional action urge — all the way — when emotion does not fit the facts
Problem Solving	—	When emotion fits the facts, solve the problem systematically
The Wave Skill	Mindfulness of Current Emotion	Observe emotion as a wave; do not fight, do not hold; let it pass
Model of Emotions	A-B-C-D chain	Vulnerability factors → Prompting event → Interpretation → Response → Aftereffects

Opposite Action — Master Table:

Emotion	Action Urge	Opposite Action
Fear	Avoid, escape	Approach; confront repeatedly (exposure)
Anger	Attack, confront	Avoid temporarily; approach with empathy and kindness
Sadness	Withdraw, be passive	Become active; engage; resume pleasurable activities
Shame (unjustified)	Hide, lower head	Go public; stand tall; maintain eye contact
Shame (justified)	Hide, punish self	Face the music; accept responsibility; apologise; repair
Guilt (justified)	Beg forgiveness	Apologise once; make amends; commit to change
Guilt (unjustified)	Over-apologise	Do not apologise; stand tall; let go
Jealousy	Demand, control, check	Do not act on jealousy; engage in other activities

EXAM PEARL

Opposite action only works when done "all the way" — posture, tone of voice, facial expression, thoughts, and actions must all be opposite. Half-hearted opposite action does not change the emotion. This is consistent with the facial feedback hypothesis and embodied cognition research.

MNEMONIC

ABC PLEASE — the "ABC" is proactive (Accumulate positives, Build mastery, Cope ahead) and the "PLEASE" is preventive (PhysicaL illness, Eating, Avoid substances, Sleep, Exercise). Together they reduce vulnerability before emotions fire.

Interpersonal Effectiveness

Skill	Acronym	Goal	Components
DEAR MAN	Describe, Express, Assert, Reinforce, Mindful, Appear confident, Negotiate	Getting what you want / saying no	Objective effectiveness
GIVE	Gentle, Interested, Validate, Easy manner	Maintaining the relationship	Relationship effectiveness
FAST	Fair, no Apologies, Stick to values, Truthful	Maintaining self-respect	Self-respect effectiveness
THINK	Think, Have empathy, Interpretations, Notice, Kindness	Reducing hostile attributions	Adolescent-specific (Rathus & Miller)

EXAM PEARL

DEAR MAN, GIVE, and FAST represent three different goals of any interpersonal interaction: getting your objective (DEAR MAN), keeping the relationship (GIVE), and keeping your self-respect (FAST). The clinical skill is knowing which to prioritise in a given situation. They are not always compatible — sometimes getting what you want costs the relationship.

MNEMONIC

DEAR MAN = the "what to do" (Describe, Express, Assert, Reinforce) + the "how to do it" (Mindful, Appear confident, Negotiate). The first four letters are content; the last three are style.

Walking the Middle Path (Adolescent DBT)

Component	Content
Dialectics	Move from "either/or" to "both/and"; find the kernel of truth in each side; no absolute truth
Validation	Six levels of validation (see below); validate the valid, not the invalid
Behaviour Change	Positive reinforcement, negative reinforcement, shaping, extinction, punishment; behavioural burst during extinction

EXAM PEARL

Walking the Middle Path is unique to adolescent DBT (Rathus & Miller, 2015). It is not in standard adult DBT. It directly addresses the three adolescent-specific dialectical dilemmas by teaching families dialectical thinking, validation skills, and behaviour-change principles.

2.7 DBT Mnemonics Master Table

Mnemonic	Stands For	Module	Use
DEAR MAN	Describe, Express, Assert, Reinforce, Mindful, Appear confident, Negotiate	Interpersonal Effectiveness	Asking for what you want; saying no
GIVE	Gentle, Interested, Validate, Easy manner	Interpersonal Effectiveness	Maintaining relationships
FAST	Fair, no Apologies, Stick to values, Truthful	Interpersonal Effectiveness	Maintaining self-respect
THINK	Think, Have empathy, Interpretations, Notice, Kindness	Interpersonal Effectiveness (adolescent)	Reducing hostile attributions
TIPP	Temperature, Intense exercise, Paced breathing, Progressive relaxation	Distress Tolerance	Rapid crisis downregulation
STOP	Stop, Take a step back, Observe, Proceed mindfully	Distress Tolerance	Impulse control
ACCEPTS	Activities, Contributing, Comparisons, Emotions, Pushing away, Thoughts, Sensations	Distress Tolerance	Distraction in crisis

Mnemonic	Stands For	Module	Use
IMPROVE	Imagery, Meaning, Prayer, Relaxation, One thing, Vacation, Encouragement	Distress Tolerance	Improving the moment
ABC PLEASE	Accumulate positives, Build mastery, Cope ahead, PhysicaL illness, Eating, Avoid substances, Sleep, Exercise	Emotion Regulation	Reducing vulnerability
WAVE	— (metaphor)	Emotion Regulation	Mindfulness of current emotion

2.8 Validation Levels 1-6

Level	Name	Description	Example
1	Unbiased listening	Full attentive presence; putting aside distractions	Putting phone down, making eye contact, staying focused
2	Accurate reflection	Paraphrasing without distortion	"So what you're saying is..."
3	Mind-reading	Articulating unspoken thoughts or feelings from nonverbal cues	"I can see this is really painful for you right now"
4	Validation by past history	Behaviour makes sense given learning history and past experiences	"Given what you've been through, of course you react this way"
5	Validation by current context	Behaviour makes sense given current circumstances; normalising	"Anyone in your situation would feel anxious right now"
6	Radical genuineness	Treating the person as a capable equal; honest, direct, warm	Giving honest feedback without being patronising

EXAM PEARL

Level 5 validation is the heart of DBT validation — it normalises the patient's response within their current context. Level 6 is the highest level and the hardest — radical genuineness means treating the patient as capable of handling truth, not as fragile. Many therapists avoid Level 6 out of their own discomfort with directness.

> EXAM STRATEGY
>
> If asked about "the most important level of validation in DBT," the answer is Level 5 (current context). If asked about "the highest level," the answer is Level 6 (radical genuineness).

2.9 Chain Analysis and Solution Analysis

Chain analysis is a detailed behavioural assessment of the sequence leading to a problem behaviour:

1. Identify the problem behaviour (specific, observable)
2. Identify the prompting event (what set it off)
3. Identify vulnerability factors (what made you susceptible today)
4. Map the chain: event → thought → emotion → body sensation → action urge → behaviour, link by link
5. Identify consequences (reinforcers maintaining the behaviour)

Solution analysis follows immediately:

1. At each link in the chain, identify which skill could have interrupted the sequence
2. Develop a prevention plan for vulnerability factors
3. Commit to specific skill use if the chain begins again

> CLINICAL ANCHOR
>
> Chain analysis is not punishment — it is collaborative detective work. Done well, patients begin to see the predictable cascade that leads to their problem behaviours and gain agency over the process. Done badly (interrogation style, blame-focused), it becomes aversive and therapy-interfering.

2.10 Diary Card

The diary card is a weekly self-monitoring tool completed daily. It tracks:

- Suicidal urges (0–5 scale)
- Self-harm urges and acts
- Misery (0–5)
- Substance use
- Emotions experienced
- Skills used (checkboxes for each skill)
- Custom targets (personalised)

The diary card is reviewed at the start of every individual therapy session. It determines which targets are addressed according to the target hierarchy.

> EXAM PEARL
>
> If the diary card shows both suicidal urges and missed skills group, suicidal urges are addressed first (life-threatening > therapy-interfering). The diary card is not optional — not completing it is itself a therapy-interfering behaviour.

2.11 Evidence Base

Study	Design	Finding
Linehan et al. (1991)	RCT, BPD women	DBT superior to TAU: fewer suicide attempts, less self-harm, fewer hospitalisations, lower dropout
Linehan et al. (2006)	RCT, BPD women	DBT superior to community treatment by experts: halved suicide attempts, reduced self-harm, ED visits, psychiatric hospitalisation
Rathus & Miller (2002)	Quasi-experimental, suicidal adolescents	DBT feasible for adolescents; fewer hospitalisations, lower dropout than TAU
Mehlum et al. (2014)	RCT, self-harming adolescents	16–week adolescent DBT superior to enhanced usual care: significant reduction in self-harm and suicidality
Linehan et al. (2015)	Skills training component analysis	DBT skills training alone (without individual therapy) reduced suicidal ideation, depression, and anxiety

3. SCHEMA THERAPY (ST)

3.1 The 18 Early Maladaptive Schemas and 5 Domains

Early Maladaptive Schemas (EMSs) are broad, pervasive patterns of memories, emotions, cognitions, and bodily sensations regarding oneself and relationships, developed in childhood when core emotional needs go unmet,

and repeated throughout life. Behaviour is NOT part of the schema itself — it is a coping response.

Five core emotional needs:

1. Secure attachment to others
2. Autonomy and competence
3. Freedom to express valid needs and emotions
4. Spontaneity and play
5. Realistic limits and self-control

MNEMONIC

MNEMONIC: DISCO for the 5 schema domains — **D**isconnection & Rejection, **I**mpaired Autonomy, Impaired Limit**S**, Other-dire**C**tedness, **O**vervigilance & Inhibition.

Full 18 EMSs Table

Domain	#	Schema	Core Belief	Typical Origins
I. Disconnection & Rejection	1	Abandonment /Instability	"People I love will leave me or be unpredictable"	Unstable, unpredictable parent; early loss
2	Mistrust/Abuse	"Others will hurt, exploit, or betray me"	Physical, sexual, emotional abuse; exploitation	
3	Emotional Deprivation	"My emotional needs will never be met"	Cold, unaffectionate parents; three subtypes: nurturance, empathy, protection	
4	Defectiveness /Shame	"I am fundamentally flawed and unlovable"	Critical, rejecting parents; compared to siblings	
5	Social Isolation/Alien ation	"I don't belong; I am different from everyone"	Family different from surroundings; not fitting in	
II. Impaired Autonomy & Performance	6	Dependence/I ncompetence	"I cannot handle life without help"	Overprotectiv e parents who did everything for child
7	Vulnerability to Harm/Illness	"Catastrophe is imminent and I cannot prevent it"	Anxious parent who modelled catastrophisin g	

Domain	#	Schema	Core Belief	Typical Origins
8	Enmeshment/ Undeveloped Self	"I have no identity apart from significant others"	Enmeshed family; parent who lived through child	
9	Failure	"I am stupid, inept, and will always fail"	Parents who criticised achievement; unrecognised ADHD	
III. Impaired Limits	10	Entitlement/Grandiosity	"I am superior; rules don't apply to me"	Permissive, overindulgent parents; OR overcompensation for deprivation
11	Insufficient Self-Control	"I cannot tolerate frustration or control impulses"	Lack of discipline or structure in childhood	
IV. Other-Directedness	12	Subjugation	"I must submit or others will punish/abandon me"	Domineering, controlling parent
13	Self-Sacrifice	"I must meet others' needs at my own expense"	Weak, needy, or ill parent; parentified child	
14	Approval-Seeking	"I need others' approval to feel worthy"	Parents valued status over emotional needs	

Domain	#	Schema	Core Belief	Typical Origins
V. Overvigilance & Inhibition	15	Negativity/Pessimism	"Things will go wrong; I must be prepared for the worst"	Pessimistic, worried parent; repeated losses
16	Emotional Inhibition	"I must suppress my emotions to avoid disapproval"	Family discouraged emotional expression	
17	Unrelenting Standards	"I must be perfect to avoid criticism"	Parents who set impossibly high standards	
18	Punitiveness	"Mistakes deserve harsh punishment"	Punitive, unforgiving parents	

EXAM PEARL

The domain distribution is 5–4–2–3–4 (easy to remember: the Impaired Limits domain has only 2 schemas — Entitlement and Insufficient Self-Control). Domain I (Disconnection & Rejection) contains the schemas most associated with personality disorders.

EXAM PEARL

Conditional schemas (Subjugation, Self-Sacrifice, Approval-Seeking, Emotional Inhibition, Unrelenting Standards) develop later as attempts to cope with unconditional schemas (the remaining 13). Example: Unrelenting Standards develops in response to Defectiveness — "If I can be perfect, I will be worthy of love."

> EXAM PEARL
>
> Behaviour is NOT part of the schema. Schemas comprise memories, emotions, cognitions, and bodily sensations. This is why different patients use different — even opposite — behaviours to cope with the same schema (surrender vs. avoidance vs. overcompensation).

3.2 Schema Modes

Modes are moment-to-moment emotional states and coping responses. While schemas are traits, modes are states. Modes are more directly accessible and targetable in session.

Schema Modes Table (Young 2003 — 10 modes)

Category	Mode	Description
Child Modes	Vulnerable Child	Feels abandoned, abused, deprived, helpless, frightened
Angry Child	Enraged about unmet needs; vents without regard for consequences	
Impulsive/Undisciplined Child	Acts impulsively; low frustration tolerance	
Happy Child	Core needs met; spontaneous, content	
Coping Modes	Compliant Surrenderer	Passive, subservient; submits to avoid conflict
Detached Protector	Emotionally withdrawn, numb; may use substances or fantasy	
Overcompensator	Counterattacks; appears grandiose, aggressive, or controlling	
Parent Modes	Punitive/Critical Parent	Punishes child for expressing needs; harsh self-criticism
Demanding Parent	Sets impossibly high expectations; pressures for perfection	
Healthy Mode	Healthy Adult	Nurtures Vulnerable Child, limits Angry/Impulsive Child, battles maladaptive modes

Arntz/Bernstein Expanded Mode Model (18 modes)

Category	Mode	Key Affect/Function
Dysfunctional Child — Vulnerability	Lonely child	Empty, alone, unlovable
Abandoned/abused child	Fear, pain, defencelessness	
Humiliated/inferior child	Shame, inferiority	
Dependent child	Overwhelmed, regressive	
Dysfunctional Child — Anger	Angry child	Frustrated, rebellious
Obstinate child	Passive resistance	
Enraged child	Uncontrolled aggression	
Dysfunctional Child — Discipline	Impulsive child	Selfish, no delay of gratification
Undisciplined child	Gives up, cannot tolerate boredom	
Dysfunctional Parent	Punitive parent	Self-loathing, harsh criticism
Demanding parent	Excessive standards, pressure	
Coping — Surrender	Compliant surrender	Passive, subservient, self-deprecating
Coping — Avoidance	Detached protector	Emotional shutdown, dissociation
Avoidant protector	Behavioural avoidance of situations	
Angry protector	Wall of anger for distance	
Detached self-soother	Addictive soothing/stimulation	

Category	Mode	Key Affect/Function
Coping — Overcompensation	Self-aggrandiser	Grandiose, denigrating
Attention-seeker	Extravagant, approval-seeking	
Perfectionistic overcontroller	Control via perfectionism	
Paranoid overcontroller	Control via vigilance/suspicion	
Bully & attack	Intimidation, hot aggression	
Conning/manipulative	Deceit for goals	
Predator	Cold, ruthless elimination of threats	
Healthy	Happy child	Core needs met, spontaneous
Healthy adult	Realistic, responsible, balanced	

EXAM PEARL

Young (2003) described 10 modes. Arntz & Jacob (2013) expanded to 18+ modes, primarily by differentiating coping modes more granularly. The expanded model is assessed via the Schema Mode Inventory (SMI; Lobbestael et al., 2010).

CLINICAL ANCHOR

Differentiating anger-related modes uses countertransference: angry child = impulsive, childlike, patient wants repair; angry protector = secondary emotion creating distance, wall-like quality; bully & attack = controlled intimidation, therapist feels threatened. Your own emotional response tells you which mode you are facing.

3.3 Three Coping Styles

Coping Style	Threat Response Analogue	Mechanism	Example (Abandonment Schema)
Surrender	Freeze	Gives in to the schema; behaves as if it were true	Selects uncommitted partners and stays
Avoidance	Flight	Avoids triggering situations, emotions, or thoughts	Avoids intimate relationships; drinks when alone
Overcompensation	Fight	Behaves as though the opposite of the schema were true	Clings, smothers; attacks partner for minor separations

MNEMONIC

MNEMONIC: SAO — Surrender (Freeze), Avoidance (Flight), Overcompensation (Fight). Same schema, three completely different behavioural presentations.

3.4 Limited Reparenting

The defining therapeutic relationship model of Schema Therapy. The therapist provides, within appropriate boundaries, what the patient needed but did not receive from parents. This is not becoming a parent — it is a consistent relational stance calibrated to the patient's unmet needs:

- Abandonment → provide stability, consistency, reliability
- Emotional Deprivation → provide warmth, empathy, guidance
- Defectiveness → be accepting, nonjudgmental; welcome flaws
- Subjugation → be non-directive; invite choices
- Entitlement → set limits firmly; hold boundaries

> EXAM PEARL
>
> Nadort et al. (2009) showed that crisis phone support outside office hours adds no benefit for BPD patients receiving Schema Therapy. This was removed from the protocol. Email contact is offered instead.

3.5 Core Techniques

Cognitive techniques: Schema validity testing, reframing evidence, coping-style pros/cons, schema flash cards, schema diaries.

Experiential techniques:

- **Imagery rescripting** — patient accesses childhood memory via affect bridge; helping figure (therapist → third party → patient's healthy adult) enters scene and rescripts the experience. NOT exposure — patient contacts emotion but does not relive full trauma.
- **Chair work** — two-chair (coping mode exploration) or three-chair (parent mode reduction) dialogues. Therapist initially models Healthy Adult; patient gradually takes over.

Behavioural techniques: Pattern-breaking assignments, social skills training, exposure with response prevention (for coping modes), positive activity scheduling.

> EXAM PEARL
>
> In imagery rescripting, the therapist does NOT ask permission to enter the scene — they simply step in. Three types of helping figures correspond to healthy adult strength: (1) therapist enters (weak HA), (2) third-party helper (intermediate), (3) patient as own HA (strong). Progression across therapy: 1 → 2 → 3.

> EXAM PEARL
>
> Imagery rescripting is NOT exposure therapy. Arntz et al. (2007) showed IR was superior to imaginal exposure on guilt, shame, and anger control in PTSD. The patient contacts the emotion but is not required to relive the full trauma.

3.6 Mode Model for BPD

Four core BPD modes:

1. **Abandoned/abused child** — intense abandonment fear, helplessness
2. **Angry/impulsive child** — rage at unfair treatment + impulsive need fulfilment
3. **Punitive parent** — extreme self-hatred (the signature BPD mode)
4. **Detached protector** — emotional shutdown, dissociation, substance use

Treatment: 2–3 years, starting at 2×/week. Phase 1 (up to 1 year): work with coping modes, build therapeutic relationship. Phase 2: imagery rescripting for trauma, chair dialogues against punitive parent. Phase 3: transfer healthy adult role, behavioural pattern-breaking.

> EXAM PEARL
>
> Giesen-Bloo et al. (2006) found psychotropic medication — particularly sedatives/benzodiazepines — significantly reduced ST effectiveness in BPD, likely by blocking emotional intensity required for experiential interventions. This is one of the few studies showing medication can impair psychotherapy outcomes.

3.7 Evidence Base

Study	Finding
Giesen-Bloo et al. (2006)	Outpatient ST superior to TFP for BPD (RCT, Archives of General Psychiatry)
Farrell et al. (2009)	Group ST effective for BPD outpatients
Nadort et al. (2009)	Crisis phone support adds no benefit; removed from protocol
Bamelis et al. (2014)	ST effective for Cluster C personality disorders and paranoid PD
Arntz et al. (2007)	IR superior to imaginal exposure for PTSD on anger, guilt, shame

4. ACCEPTANCE AND COMMITMENT THERAPY (ACT)

4.1 Theoretical Foundation

ACT (pronounced as one word, "act") is rooted in **Relational Frame Theory (RFT)**, a post-Skinnerian account of human language and cognition. RFT proposes that human suffering is largely a product of language: the ability to relate events symbolically means we can re-experience pain through words alone ("remembering" a loss), create suffering about the future ("what if I fail"), and fuse with verbal rules that restrict our behaviour ("I'm not the kind of person who...").

The fundamental problem is not the content of thoughts but our relationship to them. Attempting to control or eliminate unwanted thoughts and feelings (experiential avoidance) is itself the pathological process. ACT targets not symptoms but **psychological inflexibility** — the rigid dominance of verbal processes over direct experience, leading to behaviour that is incongruent with values.

> EXAM PEARL
>
> ACT is the only third-wave therapy built on an explicit basic science foundation (RFT). While other therapies are empirically tested treatment packages, ACT's theoretical model generates novel, testable predictions about language, cognition, and behaviour change.

4.2 Psychological Flexibility and the Hexaflex

The goal of ACT is psychological flexibility: the ability to contact the present moment fully as a conscious human being, and to change or persist in behaviour in the service of chosen values. Six core processes, arranged in a "hexaflex" model, interact to produce flexibility:

Process	Inflexibility Pole	Flexibility Pole	Description
Acceptance	Experiential avoidance	Acceptance	Willingness to have unwanted internal experiences without trying to change, escape, or avoid them
Cognitive Defusion	Cognitive fusion	Defusion	Seeing thoughts as thoughts (mental events) rather than as literal truths; stepping back from thought content
Present Moment	Dominance of past/future	Present-moment awareness	Flexible, voluntary attention to the here-and-now; mindfulness
Self-as-Context	Attachment to conceptualised self	Self-as-context	The observing self — a stable perspective from which to notice changing thoughts, feelings, and roles; "the sky, not the weather"
Values	Lack of values clarity	Values	Chosen life directions that give meaning; not goals but ongoing directions
Committed Action	Inaction/impulsivity /avoidance	Committed action	Concrete behaviours in service of values; includes willingness to fail and try again

EXAM PEARL

The hexaflex has two overarching processes: (1) Mindfulness and acceptance processes (acceptance, defusion, present moment, self-as-context) and (2) Commitment and behaviour change processes (values, committed action). The six processes are not sequential stages — they are interdependent and worked on simultaneously.

MNEMONIC

The hexaflex can be remembered as: "Accept thoughts (acceptance + defusion) while being here (present moment + self-as-context) and doing what matters (values + committed action)."

4.3 Key Techniques

Creative hopelessness: The therapist helps the patient recognise that all prior attempts to control or eliminate unwanted internal experiences have failed — not because the patient was bad at it, but because the control agenda itself is the problem. This is not nihilism; it creates openness to an alternative approach.

Defusion techniques: "I'm having the thought that I'm worthless" (not "I'm worthless"); thanking your mind for the thought; singing intrusive thoughts to a silly tune; saying the thought slowly until it becomes just sounds; "leaves on a stream" exercise.

Values clarification: Identifying what truly matters across life domains (relationships, work, health, play, spirituality). Values are not goals — they are directions. You never "achieve" a value; you move toward it.

Committed action: Specific, concrete, values-consistent behaviour patterns. Includes exposure exercises, skills building, and goal-setting — similar to traditional CBT behavioural activation but explicitly linked to values rather than mood improvement.

CLINICAL ANCHOR

ACT does not aim to reduce symptoms. It aims to increase psychological flexibility. Symptom reduction may occur as a byproduct, but it is not the goal. This is a fundamental philosophical departure from both CBT and DBT, and exam questions sometimes test this distinction.

4.4 Evidence Base

Condition	Evidence Level	Key Finding
Chronic pain	Strong (multiple RCTs)	ACT superior to TAU and comparable to CBT; improvements in disability, pain interference
Depression	Moderate (RCTs)	ACT comparable to CBT; mediational analyses support flexibility as mechanism
Anxiety disorders	Moderate (RCTs)	ACT comparable to CBT; especially effective when experiential avoidance is prominent
Psychosis	Emerging (RCTs)	ACT reduces rehospitalisation and distress associated with hallucinations
Substance use	Emerging	ACT shows promise for smoking cessation and polysubstance use
Transdiagnostic	Growing	ACT increasingly used as a transdiagnostic intervention; flexibility is dimension, not diagnosis-specific

EXAM PEARL

A-Tjak et al. (2015, meta-analysis, Behaviour Research and Therapy) found ACT superior to control conditions across all outcomes and comparable to CBT. ACT showed particular strength when experiential avoidance was the primary maintaining factor.

5. MINDFULNESS-BASED COGNITIVE THERAPY (MBCT)

5.1 Theoretical Foundation

MBCT was developed by Zindel Segal, Mark Williams, and John Teasdale specifically to prevent relapse in recurrent depression. It is not a general mindfulness programme — it has a precise theoretical rationale.

Differential activation hypothesis (Teasdale, 1988): In remitted depression, low mood can reactivate the specific cognitive patterns (negative automatic thoughts, rumination, hopelessness) that were present during the depressive episode. This reactivation occurs through associative memory networks — the more episodes a person has had, the less mood provocation is required to trigger the full depressive cognitive package. After three or more episodes, relapse can be triggered by normal sadness.

Rumination as mechanism: Rumination (repetitive, self-focused, analytical thinking about causes and consequences of distress) maintains and deepens depressive episodes. MBCT targets rumination not by changing thought content but by changing the mode of processing: from "doing mode" (goal-directed, discrepancy-monitoring, analytical) to "being mode" (present-focused, accepting, experiential).

EXAM PEARL

MBCT works by preventing the "cognitive reactivation cascade" — the process by which mild dysphoria snowballs into full relapse through rumination. It does not treat acute depression. Its primary indication is relapse prevention in patients with three or more prior episodes of depression.

EXAM PEARL

The "doing mode" vs. "being mode" distinction is central to MBCT. Doing mode monitors the gap between current state and desired state (discrepancy monitoring), which triggers more effortful processing, which deepens rumination. Being mode accepts current experience as it is, interrupting the discrepancy-monitoring loop.

5.2 The 8-Session Structure

Session	Theme	Key Practices
1	Automatic pilot	Raisin exercise (mindful eating); body scan; recognising autopilot
2	Dealing with barriers	Body scan; thoughts and feelings exercise; identifying wandering mind
3	Mindfulness of the breath	Sitting meditation (breath focus); 3-minute breathing space; seeing/hearing exercise
4	Staying present	Sitting meditation (breath + body); recognising aversion; "thoughts are not facts"
5	Allowing/letting be	Sitting meditation (sounds, thoughts); exploring difficulty; working with aversion
6	Thoughts are not facts	Sitting meditation (choiceless awareness); moods and alternative viewpoints
7	How can I best take care of myself?	Links between activity and mood; nourishing vs. depleting activities; relapse signature; action plan
8	Using what has been learned	Review of course; body scan; maintaining practice; relapse prevention plan

The 3-Minute Breathing Space: The signature MBCT micro-practice, taught from Session 3 and used throughout.

- Minute 1: Awareness — "What is my experience right now?" (thoughts, feelings, body sensations)
- Minute 2: Gathering — Narrow attention to the breath
- Minute 3: Expanding — Expand awareness to the whole body and the present moment

> EXAM PEARL
>
> The 3-minute breathing space is used both as a scheduled practice (3× daily) and as a "coping" breathing space (when noticing the beginnings of rumination or mood shift). It is the bridge between formal meditation and daily life — the most clinically important MBCT practice.

> CLINICAL ANCHOR
>
> MBCT is delivered as a group programme (12–15 participants) over 8 weeks with daily homework (45 minutes of formal practice + informal practice). It is not individual therapy. The group format is essential — it normalises the experience and creates a community of practice.

5.3 Evidence Base

Study	Finding
Teasdale et al. (2000)	MBCT reduced relapse by 44% in patients with 3+ prior episodes vs. TAU; no benefit for 1–2 episodes
Ma & Teasdale (2004)	Replicated: MBCT halved relapse risk for 3+ episodes; specific to those with childhood adversity
Kuyken et al. (2008)	MBCT comparable to maintenance antidepressant medication for relapse prevention
Kuyken et al. (2015)	MINDFUL trial (RCT, n=424): MBCT with tapering antidepressants non-inferior to maintenance antidepressants over 24 months
NICE (2009, updated 2022)	MBCT recommended for recurrent depression (3+ episodes) as relapse prevention

EXAM PEARL

MBCT is effective specifically for patients with three or more prior depressive episodes. Teasdale et al. (2000) found NO benefit for patients with only 1–2 episodes. The differential activation hypothesis explains this: with fewer episodes, relapse is driven more by life events than by cognitive reactivation, so a cognitive intervention adds less.

6. COMPASSION-FOCUSED THERAPY (CFT)

6.1 Theoretical Foundation

CFT was developed by Paul Gilbert for patients with high shame and self-criticism — particularly those who can generate alternative thoughts cognitively but cannot feel reassured by them. Gilbert's key insight: understanding

intellectually that you are not worthless does not reduce shame if the emotional system generating shame is not also addressed.

6.2 Three Emotion Regulation Systems

System	Function	Affect	Neurobiology	Pathology When Dominant
Threat/Protection	Detect and respond to danger	Anxiety, anger, disgust, shame	Amygdala, HPA axis, SNS	Anxiety disorders, PTSD, OCD, shame-based conditions
Drive/Resource-Seeking	Pursue goals, rewards, and resources	Excitement, anticipation, pleasure	Dopamine circuits, nucleus accumbens	Mania, addiction, narcissism, workaholism
Soothing/Affiliation	Calm, connect, feel safe	Contentment, safety, connection	Oxytocin, endorphins, vagal tone	Inadequately developed in high-shame patients

> EXAM PEARL
>
> The core pathology in CFT's model is an underdeveloped soothing/affiliation system combined with an overactive threat system. Many patients oscillate between threat (anxiety, self-criticism) and drive (achievement, perfectionism) without ever accessing soothing. Traditional CBT addresses the threat system (cognitive restructuring of threats) but does not build the soothing system.

6.3 Compassionate Mind Training

CFT uses specific exercises to activate and strengthen the soothing/affiliation system:

- **Compassionate imagery:** Creating a detailed image of an ideal compassionate figure (wise, strong, warm, non-judgmental) and practising

receiving compassion from this figure

- **Compassionate self:** Developing an inner compassionate voice — not just correcting distorted thoughts but speaking to oneself with warmth, understanding, and encouragement
- **Compassion-focused exposure:** Approaching feared situations while deliberately activating the soothing system rather than relying on threat-system coping
- **Fears of compassion:** Many patients actively resist self-compassion (fear of self-indulgence, grief about unmet needs, belief they do not deserve it). These fears are directly addressed as therapy targets

CLINICAL ANCHOR

CFT is especially indicated when standard CBT cognitive restructuring produces intellectual change without emotional change — "I know I'm not worthless but I still feel it." This gap between head and heart is the signal that the soothing system needs building, not just the threat system needs correcting.

EXAM PEARL

Gilbert identified three "flows" of compassion: compassion for others, compassion from others, and self-compassion. Many patients can feel compassion for others but cannot receive it or generate it for themselves. The "flows" help identify where the blockage lies.

7. COMPARISON TABLE

7.1 Third-Wave Therapies Compared

Dimension	DBT	Schema Therapy	ACT	MBCT	CFT
Founder	Linehan (1993)	Young (1990/2003)	Hayes (1999)	Segal, Williams, Teasdale (2002)	Gilbert (2005)
Theoretical basis	Biosocial model + dialectics + Zen	Unmet childhood needs → EMSs; attachment theory	RFT; functional contextualism	Differential activation; cognitive vulnerability to relapse	Evolutionary psychology; three emotion regulation systems
Core concept	Acceptance + change	Early Maladaptive Schemas and modes	Psychological flexibility	Mindful awareness preventing cognitive reactivation	Compassion as antidote to shame
Primary target	Emotional dysregulation; suicidality	Characterological patterns; personality disorders	Experiential avoidance; psychological inflexibility	Depressive relapse (3+ episodes)	High shame; self-criticism
Format	Individual + group + phone + consult team	Individual (1–2×/week); some group adaptations	Individual or group	8–session group programme	Individual or group

Dimension	DBT	Schema Therapy	ACT	MBCT	CFT
Duration	1 year (standard); 24 weeks (adolescent)	1–3 years (personality disorders)	8–16 sessions typically	8 weeks	12–20 sessions typically
Therapeutic relationship	Dialectical: validation + problem-solving	Limited reparenting + empathic confrontation	Functional; therapist models flexibility	Group facilitator; teacher	Compassionate; models warmth/non-judgment
Key techniques	Skills training, chain analysis, diary card, validation	Imagery rescripting, chair work, flash cards, pattern-breaking	Defusion, values clarification, creative hopelessness, committed action	Mindfulness meditation, body scan, 3–minute breathing space	Compassionate imagery, compassionate self, fears of compassion
Mindfulness role	Core module; gateway skill	Not central; used in some adaptations	Present-moment awareness (1 of 6 processes)	Central and defining	Embedded in compassionate awareness practices
Evidence level	Strong (multiple RCTs for BPD, self-harm, suicidality)	Strong for BPD (Giesen-Bloo 2006); growing for other PDs	Moderate-strong across multiple conditions	Strong for recurrent depression relapse prevention	Growing; moderate evidence for shame-based presentations
Primary populations	BPD, suicidal/self-harming, emotion dysregulation	Personality disorders, chronic Axis I, treatment-resistant	Chronic pain, anxiety, depression, transdiagnostic	Recurrent depression (3+ episodes)	High shame, self-criticism, complex trauma

EXAM STRATEGY

When comparing third-wave therapies, the key discriminating features are: (1) DBT = acceptance + change dialectic + skills training, (2) ST = childhood schemas + limited reparenting + experiential techniques, (3) ACT = psychological flexibility + defusion + values, (4) MBCT = mindfulness + relapse prevention for recurrent depression, (5) CFT = three emotion systems + compassion training for shame. If the question describes a patient with high shame who can think rationally but not feel it, the answer is CFT. If recurrent depression with rumination, MBCT. If chronic BPD with suicidality, DBT. If deep characterological patterns from childhood, ST.

EXAM PEARL

All third-wave therapies share: (1) acceptance-based stance, (2) emphasis on process over content, (3) attention to the therapeutic relationship, (4) mindfulness (in varying degrees), (5) focus on function rather than form of behaviour. They differ in: theoretical basis, primary target population, specific techniques, and the role of mindfulness.

CLINICAL ANCHOR

In clinical practice, these therapies are often integrated rather than delivered in pure form. DBT skills are used in Schema Therapy as "temporary bridges" for self-harm and dissociation. ACT's defusion techniques complement CBT's cognitive restructuring. CFT's compassionate imagery can be incorporated into Schema Therapy's limited reparenting framework. The clinical skill is knowing which approach best fits the patient's presentation and phase of treatment.

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02

Clinical Quick Reference

Third-Wave Therapies — Weave Psychotherapy Vol. 6

D6: Third-Wave Therapies — Quick Reference

1. Third-Wave Therapies at a Glance

Wave	Era	Core Idea	Key Therapies
First	1950s-1970s	Modify observable behaviour via conditioning	Systematic desensitisation, token economies, exposure
Second	1970s-1990s	Identify and change distorted cognitions	CBT (Beck), REBT (Ellis)
Third	1990s-present	Change relationship to thoughts; acceptance + mindfulness + values	DBT, ST, ACT, MBCT, CFT, MCT, FAP, BA

Third-wave shared features: Acceptance over control, mindfulness, values-driven behaviour, therapeutic relationship as active ingredient, metacognition, contextual sensitivity.

2. DBT Skills Master Table

Core Mindfulness

Skill	Type	Description
Wise Mind	State of Mind	Synthesis of Emotional Mind and Reasonable Mind; intuitive knowing
Observe	"What" Skill	Notice experience without words; "Teflon mind"
Describe	"What" Skill	Put words on observations; stick to facts
Participate	"What" Skill	Fully engage in activity; "in the zone"
Non-judgmentally	"How" Skill	Notice without evaluating as good/bad
One-mindfully	"How" Skill	Do one thing at a time
Effectively	"How" Skill	Do what works; let go of "right" vs. "wrong"

Distress Tolerance — Crisis Survival

Skill	Components	Use
TIPP	Temperature (dive reflex), Intense exercise, Paced breathing, Progressive relaxation	Red-zone rapid downregulation
STOP	Stop, Take a step back, Observe, Proceed mindfully	Impulse control before acting
ACCEPTS	Activities, Contributing, Comparisons, Emotions, Pushing away, Thoughts, Sensations	Distraction (done mindfully)
Self-Soothe	Vision, Hearing, Touch, Smell, Taste, Movement	Sensory comfort
IMPROVE	Imagery, Meaning, Prayer, Relaxation, One thing, Vacation, Encouragement	In-the-moment coping
Pros and Cons	Weigh acting on urge vs. using skills; label short-term/long-term	Crisis decision-making

Distress Tolerance — Reality Acceptance

Skill	Core Principle
Radical Acceptance	Accept reality as it is; Pain + Non-acceptance = Suffering
Turning the Mind	Repeated choice to accept; not one decision but ongoing practice
Willingness vs. Willfulness	Willingness = doing what is needed; Willfulness = refusing to engage
Half-Smile	Subtle facial relaxation; behavioural cue for acceptance

Emotion Regulation

Skill	Description
ABC PLEASE	Accumulate positives, Build mastery, Cope ahead + PhysicaL illness, Eating, Avoid substances, Sleep, Exercise
Check the Facts	Is the emotion fitting the facts? Is the interpretation accurate?
Opposite Action	Act opposite to urge — all the way — when emotion does not fit facts
Problem Solving	When emotion fits facts, solve the problem systematically
The Wave	Observe emotion as a wave; do not fight, do not hold; let it pass
Model of Emotions	Vulnerability → Event → Interpretation → Response → Aftereffects

Opposite Action Quick Card:

Emotion	Urge	Opposite
Fear	Avoid	Approach; confront repeatedly
Anger	Attack	Avoid; approach with empathy
Sadness	Withdraw	Get active; engage
Shame (unjustified)	Hide	Go public; stand tall
Shame (justified)	Hide	Accept responsibility; repair
Guilt (justified)	Beg	Apologise once; make amends
Guilt (unjustified)	Over-apologise	Do not apologise; let go
Jealousy	Control	Do not act on it; engage elsewhere

Interpersonal Effectiveness

Skill	Goal	Components
DEAR MAN	Get what you want / say no	Describe, Express, Assert, Reinforce, Mindful, Appear confident, Negotiate
GIVE	Maintain relationship	Gentle, Interested, Validate, Easy manner
FAST	Maintain self-respect	Fair, no Apologies, Stick to values, Truthful
THINK	Reduce hostile attributions (adolescent)	Think, Have empathy, Interpretations, Notice, Kindness

Walking the Middle Path (Adolescent DBT Only)

Component	Key Points
Dialectics	"Both/and" not "either/or"; find kernel of truth; no absolute truth
Validation	6 levels; validate the valid, not the invalid
Behaviour Change	Reinforcement, shaping, extinction; expect behavioural burst

3. DBT Mnemonics Collection

Mnemonic	Full Form	Module
DEAR MAN	Describe, Express, Assert, Reinforce, Mindful, Appear confident, Negotiate	IE
GIVE	Gentle, Interested, Validate, Easy manner	IE
FAST	Fair, no Apologies, Stick to values, Truthful	IE
THINK	Think, Have empathy, Interpretations, Notice, Kindness	IE (adolescent)
TIPP	Temperature, Intense exercise, Paced breathing, Progressive relaxation	DT
STOP	Stop, Take a step back, Observe, Proceed mindfully	DT
ACCEPTS	Activities, Contributing, Comparisons, Emotions, Pushing away, Thoughts, Sensations	DT
IMPROVE	Imagery, Meaning, Prayer, Relaxation, One thing, Vacation, Encouragement	DT
ABC PLEASE	Accumulate positives, Build mastery, Cope ahead, PhysicaL illness, Eating, Avoid substances, Sleep, Exercise	ER
WAVE	(metaphor — emotion as wave)	ER

Module key: IE = Interpersonal Effectiveness, DT = Distress Tolerance, ER = Emotion Regulation

4. Schema Therapy — 18 EMSs Table

#	Domain	Schema	Core Belief
1	I. Disconnection & Rejection	Abandonment/Instability	"People I love will leave or be unpredictable"
2	Mistrust/Abuse	"Others will hurt, exploit, or betray me"	
3	Emotional Deprivation	"My emotional needs will never be met"	
4	Defectiveness/Shame	"I am fundamentally flawed and unlovable"	
5	Social Isolation	"I don't belong; I am different"	
6	II. Impaired Autonomy	Dependence/Incompetence	"I cannot handle life without help"
7	Vulnerability to Harm	"Catastrophe is imminent and unpreventable"	
8	Enmeshment	"I have no identity apart from others"	
9	Failure	"I am stupid, inept, and will always fail"	
10	III. Impaired Limits	Entitlement/Grandiosity	"I am superior; rules don't apply to me"
11	Insufficient Self-Control	"I cannot tolerate frustration or control impulses"	

#	Domain	Schema	Core Belief
12	IV. Other-Directedness	Subjugation	"I must submit or face punishment/abandonment"
13	Self-Sacrifice	"I must meet others' needs at my expense"	
14	Approval-Seeking	"I need approval to feel worthy"	
15	V. Overvigilance & Inhibition	Negativity/Pessimism	"Things will go wrong"
16	Emotional Inhibition	"I must suppress emotions"	
17	Unrelenting Standards	"I must be perfect"	
18	Punitiveness	"Mistakes deserve harsh punishment"	

Domain distribution: 5–4–2–3–4

Five core emotional needs: (1) Secure attachment, (2) Autonomy and competence, (3) Freedom to express needs/emotions, (4) Spontaneity and play, (5) Realistic limits and self-control

Three coping styles: Surrender (Freeze)	Avoidance (Flight)	Overcompensation (Fight)

5. Schema Modes Table

Young (2003) — Original 10 Modes

Category	Mode
Child	Vulnerable Child, Angry Child, Impulsive/Undisciplined Child, Happy Child
Coping	Compliant Surrenderer, Detached Protector, Overcompensator
Parent	Punitive Parent, Demanding Parent
Healthy	Healthy Adult

Arntz & Jacob (2013) — Expanded 18+ Modes

Category	Modes
Child — Vulnerability	Lonely, Abandoned/abused, Humiliated/inferior, Dependent
Child — Anger	Angry, Obstinate, Enraged
Child — Discipline	Impulsive, Undisciplined
Parent	Punitive, Demanding
Coping — Surrender	Compliant surrender
Coping — Avoidance	Detached protector, Avoidant protector, Angry protector, Detached self-soother
Coping — Overcompensation	Self-aggrandiser, Attention-seeker, Perfectionistic overcontroller, Paranoid overcontroller, Bully & attack, Conning/manipulative, Predator
Healthy	Happy child, Healthy adult

BPD Mode Model (4 core modes)

1. Abandoned/abused child (abandonment fear, helplessness)

2. Angry/impulsive child (rage, impulsive need fulfilment)
3. Punitive parent (extreme self-hatred — the signature BPD mode)
4. Detached protector (emotional shutdown, dissociation)

NPD Mode Model (3 core modes)

1. Lonely child (Emotional Deprivation + Defectiveness)
2. Self-aggrandiser (Entitlement as overcompensation)
3. Detached self-soother (avoidance through addictive/compulsive activities)

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6. ACT Hexaflex Quick Card

Process	Inflexible Pole	Flexible Pole	In One Sentence
Acceptance	Experiential avoidance	Acceptance	Have unwanted experiences without trying to escape them
Defusion	Cognitive fusion	Defusion	See thoughts as thoughts, not as facts
Present Moment	Past/future dominance	Present awareness	Flexible attention to the here-and-now
Self-as-Context	Conceptualised self	Observing self	"I am the sky, not the weather"
Values	Lack of clarity	Chosen directions	What truly matters across life domains
Committed Action	Inaction/avoidance	Values-consistent action	Concrete behaviours serving values

Two overarching clusters:

- Mindfulness and acceptance: Acceptance + Defusion + Present Moment + Self-as-Context

- Commitment and change: Values + Committed Action

Key ACT techniques: Creative hopelessness, defusion exercises ("I'm having the thought that..."), values clarification, committed action patterns, "leaves on a stream" exercise.

Core distinction from CBT: ACT does not aim to change thought content. It aims to change the relationship to thoughts. Symptom reduction is a byproduct, not a goal.

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7. MBCT Session Structure

Session	Theme	Key Practice
1	Automatic pilot	Raisin exercise; body scan
2	Dealing with barriers	Body scan; wandering mind
3	Mindfulness of breath	Sitting meditation; 3-minute breathing space
4	Staying present	Breath + body meditation; "thoughts are not facts"
5	Allowing/letting be	Sounds and thoughts meditation; working with aversion
6	Thoughts are not facts	Choiceless awareness; alternative viewpoints
7	Self-care	Nourishing vs. depleting activities; relapse signature; action plan
8	Maintaining practice	Review; body scan; relapse prevention plan

3-Minute Breathing Space:

- Minute 1: Awareness — "What is my experience right now?"

- Minute 2: Gathering — Narrow to breath
- Minute 3: Expanding — Whole body and present moment

Mechanism: Doing mode (discrepancy monitoring → rumination) → Being mode (present-focused acceptance)

Indication: Relapse prevention for recurrent depression (3+ episodes). NOT for acute depression. No benefit for 1–2 episodes.

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8. Comparison: All Third-Wave Therapies

Dimension	DBT	Schema Therapy	ACT	MBCT	CFT
Founder	Linehan	Young	Hayes	Segal/Williams/Teasdale	Gilbert
Year	1993	1990/2003	1999	2002	2005
Core concept	Acceptance + change	EMSs + modes	Psychological flexibility	Mindful awareness vs. relapse	Compassion vs. shame
Primary target	BPD, suicidality	Personality disorders	Experiential avoidance	Recurrent depression	High shame, self-criticism
Theory base	Biosocial + dialectics	Attachment + unmet needs	RFT	Differential activation	Evolutionary psychology
Format	4 modes (individual + group + phone + consult)	Individual (1–2×/week)	Individual or group	8–session group	Individual or group
Duration	1 year (24 weeks adolescent)	1–3 years	8–16 sessions	8 weeks	12–20 sessions
Relationship	Dialectical: validate + push	Limited reparenting + empathic confrontation	Models flexibility	Group facilitator	Models compassion
Mindfulness	Core module	Not central	1 of 6 processes	Central/defining	Embedded

Dimension	DBT	Schema Therapy	ACT	MBCT	CFT
Evidence	Strong (BPD, self-harm)	Strong (BPD); growing (other PDs)	Moderate-strong (transdiagnostic)	Strong (recurrent depression)	Growing
Key technique	Skills + chain analysis + diary card	IR + chair work + flash cards	Defusion + values + committed action	Meditation + 3-min breathing	Compassionate imagery

9. Evidence Snapshot

Therapy	Landmark Study	Key Finding
DBT	Linehan et al. (1991) Arch Gen Psychiatry	First RCT: DBT superior to TAU for BPD; fewer suicide attempts, less self-harm
DBT	Linehan et al. (2006)	DBT halved suicide attempts vs. community experts
DBT (adolescent)	Mehlum et al. (2014) JAACAP	Adolescent DBT superior to enhanced usual care for self-harm
ST	Giesen-Bloo et al. (2006) Arch Gen Psychiatry	ST superior to TFP for BPD; higher recovery, lower dropout
ST	Nadort et al. (2009)	No benefit from crisis phone support; removed from protocol
ST	Bamelis et al. (2014) Am J Psychiatry	ST effective for Cluster C PDs and paranoid PD
ACT	A-Tjak et al. (2015) Behav Res Ther	Meta-analysis: ACT superior to controls, comparable to CBT
MBCT	Teasdale et al. (2000) JCCP	MBCT reduced relapse 44% for 3+ episodes; no benefit for 1–2
MBCT	Kuyken et al. (2015) Lancet	MBCT non-inferior to maintenance antidepressants
CFT	Gilbert (2010, 2014)	Growing RCT evidence for shame-based presentations

Medication interaction: Giesen-Bloo et al. (2006) — sedatives/benzodiazepines significantly reduced ST effectiveness in BPD (blocks emotional processing needed for IR).

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10. Viva Questions

Q1. What distinguishes third-wave therapies from second-wave CBT?

Third-wave therapies change the patient's relationship to thoughts rather than the content of thoughts. They emphasise acceptance over control, incorporate mindfulness, attend to values and meaning, use the therapeutic relationship as an active change mechanism, and work at the metacognitive or process level. Second-wave CBT focuses on identifying and modifying distorted cognitions through logical analysis and behavioural experiments.

Q2. Describe the biosocial model in DBT.

Chronic emotional dysregulation arises from a transactional interaction between biological vulnerability (high sensitivity, high reactivity, slow return to baseline) and a pervasively invalidating environment (punishing emotions, reinforcing only extremes, oversimplifying solutions). The model is transactional — the child's sensitivity shapes the environment's response and vice versa, creating escalating cycles. The model is de-shaming: problems are not the patient's fault, but they are their responsibility.

Q3. What is the DBT target hierarchy and why does it matter?

In Stage 1 individual therapy: (1) life-threatening behaviours, (2) therapy-interfering behaviours, (3) quality-of-life-interfering behaviours, (4) increasing skills. The hierarchy is absolute — if suicidal behaviour occurred this week, it is addressed before everything else, regardless of what the patient wants to discuss. This prevents avoidance of lethal risk and ensures systematic progress.

Q4. Name all six DBT validation levels.

(1) Unbiased listening, (2) Accurate reflection, (3) Mind-reading (articulating unspoken experiences), (4) Validation by past history, (5) Validation by current context (the heart of DBT validation — normalising the response), (6) Radical genuineness (treating the person as a capable equal). Level 5 is the most important clinically; Level 6 is the highest and most challenging.

Q5. List the 5 schema domains and give one schema from each.

(1) Disconnection & Rejection — e.g., Abandonment; (2) Impaired Autonomy & Performance — e.g., Dependence/Incompetence; (3) Impaired Limits — e.g., Entitlement; (4) Other-Directedness — e.g., Self-Sacrifice; (5) Overvigilance & Inhibition — e.g., Unrelenting Standards. Distribution: 5–4–2–3–4 schemas per domain.

Q6. Explain the difference between schemas and modes in Schema Therapy.

Schemas are traits — stable, pervasive patterns of memories, emotions, cognitions, and bodily sensations. Modes are states — the moment-to-moment emotional states and coping responses currently active. A patient may have the Abandonment schema as a trait but fluctuate between the Vulnerable Child mode, Detached Protector mode, and Angry Child mode depending on the situation. Modes are more directly accessible in session and more directly targetable.

Q7. What is imagery rescripting and how does it differ from exposure therapy?

In imagery rescripting, the patient accesses a childhood memory via an affect bridge, then a helping figure (therapist, third party, or patient's own healthy adult) enters the scene and changes the outcome — protecting the child, confronting the perpetrator, meeting unmet needs. Unlike exposure therapy, the patient does NOT relive the full trauma. They contact the emotion just enough to activate the memory, then the memory is actively rescripted. Arntz et al. (2007) showed IR was superior to imaginal exposure on guilt, shame, and anger in PTSD.

Q8. Describe the ACT hexaflex and its six processes.

The hexaflex represents six processes that interact to produce psychological flexibility: (1) Acceptance — willingness to have unwanted experiences, (2) Cognitive defusion — seeing thoughts as mental events rather than truths, (3) Present-moment awareness — flexible attention to the here-and-now, (4) Self-as-context — the observing self ("the sky, not the weather"), (5) Values — chosen life directions, (6) Committed action — concrete behaviours serving values. Two clusters: mindfulness/acceptance (processes 1–4) and commitment/change (processes 5–6).

Q9. Why does MBCT only work for patients with 3+ depressive episodes?

MBCT targets the differential activation cascade: in patients with 3+ episodes, mild dysphoria can reactivate the full depressive cognitive package (negative thoughts, rumination, hopelessness) through associative memory networks. After multiple episodes, the threshold for reactivation drops so low that normal sadness triggers relapse. MBCT trains patients to notice early warning signs and shift from "doing mode" (rumination) to "being mode" (acceptance). In patients with 1–2 episodes, relapse is driven more by life events than cognitive reactivation, so the mechanism MBCT targets is not yet established.

Q10. What are the three emotion regulation systems in CFT?

(1) Threat/Protection system — detects and responds to danger; produces anxiety, anger, shame; driven by amygdala and HPA axis. (2) Drive/Resource-seeking system — pursues goals and rewards; produces excitement, anticipation; driven by dopamine. (3) Soothing/Affiliation system — calms and connects; produces contentment, safety; driven by oxytocin and vagal tone. In patients with high shame, the soothing system is underdeveloped while the threat system is overactive. CFT builds the soothing system through compassionate imagery and compassionate mind training.

Q11. Compare the therapeutic relationship across DBT, ST, and ACT.

DBT: dialectical — the therapist balances validation (acceptance) with problem-solving (change); warm but also irreverent; consultation team prevents burnout. ST: limited reparenting — the therapist provides within boundaries what the patient needed from parents (warmth for deprivation, stability for abandonment, limits for entitlement) + empathic confrontation. ACT: the therapist models psychological flexibility; therapist willingness to be present with the patient's pain without trying to fix it; relationship is functional rather than corrective.

Q12. A patient with BPD is being considered for psychotherapy. Compare how DBT and Schema Therapy would each approach the case.

DBT: Biosocial formulation; Stage 1 targets (safety first); individual therapy + skills group + phone coaching + consult team; diary card; chain analysis for self-harm; TIPP for crisis; opposite action for shame; DEAR MAN for interpersonal needs; 1–year commitment. ST: Mode formulation (abandoned child, angry child, punitive parent, detached protector); limited reparenting to build trust (Phase 1, up to 1 year on coping modes); imagery rescripting for childhood trauma (Phase 2); chair work against punitive parent; transfer of healthy adult role (Phase 3); 2–3 years, 2×/week. Key difference: DBT prioritises behavioural stabilisation and skills; ST prioritises emotional processing and schema healing. In practice, DBT skills are often used as a "bridge" in early ST when self-harm or dissociation is present.

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